Vinorelbine intravenous (IV) · non-small-cell lung cancer

Treatment scope · NCCP 00232 Lung monotherapy context only. The breast guide and its evidence remain separate. Discuss the exact disease setting and alternatives with the treating team.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Vinorelbine intravenous (IV) · non-small-cell lung cancer · support plan

Active medicines: Vinorelbine

Emetogenic risk: Minimal IV risk

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Individual antiemetic plan differs by route; list routine and rescue medicines separately.
- Discuss constipation prevention, hydration/diet where appropriate and prescribed laxatives; regular opioid use increases the need for a bowel plan.
- No routine steroid premedication, granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibacterial, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is mandated by these National Cancer Control Programme (NCCP) monotherapy documents. Consider additional prevention based on individual risk/local policy; do not create a generic automatic prescription.
- Gastric protection is a consideration for high gastrointestinal-bleed risk, not a universal drug.
- Confirm strong CYP3A4 inhibitors/inducers, P-gp modulators, phenytoin, itraconazole, warfarin and live vaccines. Yellow fever vaccine is contraindicated. Avoid grapefruit per NCCP; do not self-start supplements.
- No routine prophylaxis required by NCCP minimal-risk pathway.
- Prescriber selects an as-needed medicine according to symptoms, constipation, QT/interactions and local policy. Reassess before future doses if nausea/vomiting occurs.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00232 v5 · p5 · NCCP antiemetic guidance v6 · minimal IV pathway

Sources checked 2026-11-02 · SACTCheck v0.84.29 · Regimen-level drafts; prescriber and oncology-pharmacy verification required